



Obstetrical Emergency

Assessment

History:

- Past medical history
- Hypertension meds
- Due date or LMP
- If known high risk pregnancy
- Prenatal care
- Prior pregnancies / births
- Gravida / Para

Signs & Symptoms:

- Vaginal bleeding
- Abdominal pain
- Seizures
- Hypertension
- Severe headache
- Visual changes
- Edema of the hands and face

Differential:

- Pre-eclampsia / Eclampsia
- Placenta previa
- Placenta abruptio
- Spontaneous abortion

Clinical Management Options

P	P	P	P	P	P
L	L	L	L	L	L
1	2	3	4	5	6

- [Oxygen](#), target SpO₂ to 92-96%
- [Vascular access](#) with [Isotonic Crystalloid](#) titrated to effect for vaginal hemorrhage
- [Tranexamic Acid](#) for hypotension due to significant hemorrhage
- Seizure: [Midazolam](#)
- Suspected Eclampsia: [Magnesium Sulfate](#)
- Monitoring & Interpretation of ECG and EtCO₂

Consult Online Medical Control As Needed

Pearls:

- Refer to drug formulary charts for all medication dosing for both adults and pediatric patients.
- **Eclamptic seizures may occur up to 2 months post-partum. Always consider in pregnant/recently pregnant seizing patient.**
- Severe headache, vision changes, edema, or RUQ pain may indicate preeclampsia.
- In the setting of pregnancy, hypertension is defined as a SBP greater than >140 or a DBP > 90, or relative increase of 30 systolic and 20 diastolic from the patient's normal (pre-pregnancy) blood pressure.
- Ask patient to quantify bleeding - number of pads used per hour.
- Any pregnant patient involved in a MVC should be seen immediately by a physician for evaluation and fetal monitoring in a [Trauma Center](#).
- Magnesium may cause hypotension and decreased respiratory drive, monitor closely.
- If > 20 weeks, consider left lateral position.
- Ultrasound, if available, for PL-5 for fetal heart tones and movement.